



Patient Name : **SMITAPRAGYAN RAIGURU**
Age / Sex : 27 years / Female
Ref. Doctor : SELF
Client Name : Medibuddy
Sample ID : 8674903



Patient ID / Billing ID : 806295 / 878879
Specimen Collected by : Medibuddy
Sample Receiving Time : 14 Mar, 2026, 07:13 a.m.
Registration On : 14 Mar, 2026, 07:13 a.m.
Reported On : 14 Mar, 2026, 11:19 a.m.
Date of Birth : 07-09-1998

BIOCHEMISTRY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
<u>GLUCOSE - FASTING</u>				
Glucose Fasting (Plasma)	84.95	mg/dl	Non-Diabetic(Fasting) ≤100 Pre-Diabetic : 100 - 125 Diabetic : >126	Hexokinase

Interpretation :

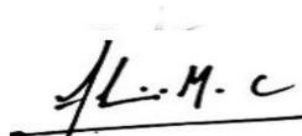
Fasting Blood Sugar more than 126 mg/dl on more than one occasion can indicate Diabetes Mellitus.

Comments: Kindly Correlate Clinically.

****END OF REPORT****

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Shruthi Sreenath
Consultant Biochemist


Dr. Ashwini M.C
Consultant Pathologist



These tests were performed at Predlabs Private Limited, Bengaluru

#360, Sri Sai Padma Arcade, 2nd Floor,
Ramagondanahalli, Varthur Main Road,
Whitefield, Bengaluru 560066





Patient Name : **SMITAPRAGYAN RAIGURU**

Age / Sex : 27 years / Female

Ref. Doctor : SELF

Client Name : Medibuddy

Sample ID : 8674901

Patient ID / Billing ID : 806295 / 878879

Specimen Collected by : Medibuddy

Sample Receiving Time : 14 Mar, 2026, 07:13 a.m.

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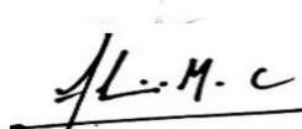
BIOCHEMISTRY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
<u>GLYCOSYLATED HEMOGLOBIN - HBA1C</u>				
Sample Type : EDTA(WB)				
HbA1c	5.26	%	Below 5.7% - Normal Value/ Non diabetic 5.7% - 6.4% pre-diabetic/ 6.5% or higher Diagnostic of Diabetic	HPLC Method
AVERAGE BLOOD GLUCOSE (ABG)	104.26	mg/dl	-	
Technology	CALCULATED			
Comments :	Please correlate with clinical conditions.			

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Date of Birth : 07-09-1998

HAEMATOLOGY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
COMPLETE BLOOD COUNT - CBC				
Sample Type : EDTA(WB)				
Haemoglobin	11.5	g/dL	12-16	Cyanide Free Colorimetric
Total WBC Count	8.18	$10^3 / \mu\text{L}$	4-11	Elec. Impedance & Optical
RBC	4.14	$10^6 / \mu\text{L}$	3.8-4.8	Elec. Impedance & Optical
Haematocrit/Packed Cell Volume	35	%	36-46	Calculated
MCV	84.3	fL	82-100	Measured
MCH	27.8	pg	26 - 32	Calculation
MCHC	33	g/dL	31.5-34.5	Calculation
RDW-SD	42.8	fL	36 - 56	Calculation
RDW-CV	14.5	%	11.5 - 14.5	Calculation
Platelet Count	150	$10^3 / \mu\text{L}$	150-450	Elec. Impedance / Optical

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DIFFERENTIAL COUNT

Neutrophils	54.6	%	40 - 75	Fluorescence Flowcytometry
Lymphocytes	32.4	%	20 - 45	Fluorescence Flowcytometry
Monocytes	8.5	%	1-10	Fluorescence Flowcytometry
Eosinophil	3.8	%	1 - 7	Fluorescence Flowcytometry
Basophils	0.7	%	0 - 1	Fluorescence Flowcytometry
Absolute Neutrophils Count	4.47	10 ³ /uL	2-8	Calculated
Absolute Lymphocyte Count	2.65	10 ³ /uL	1-5	Calculated
Absolute Eosinophil Count	0.31	10 ³ /uL	0.02 - 0.5	Calculated
Absolute Basophils Count	0.06	10 ³ /uL	0.01-0.5	Calculated
Absolute Monocyte Count	0.70	10 ³ /uL	0.2 - 1.0	Calculated

Comments : Kindly Correlate Clinically.

Remarks

Outside samples to be correlated with other clinical findings.

Disclaimer:-

C - Stands for Critical value *

****END OF REPORT****

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HAEMATOLOGY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
<u>BLOOD PICTURE - PERIPHERAL SMEAR EXAMINATION</u>				
RBC :	Normocytic Normochromic.			
WBC :	Normal in count, distribution, and morphology.			
Platelets:	Adequate on Smear			
Haemoparasite:	Not seen in present smears studied.			
Impression:	Normocytic Normochromic Blood Picture.			

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Age / Sex : 27 years / Female
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Client Name : Medibuddy
Sample ID : MB2029457

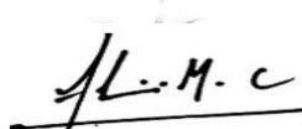


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Specimen Collected by : Medibuddy
Sample Receiving Time : 14 Mar, 2026, 07:13 a.m.
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CLINICAL PATHOLOGY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
CUE - COMPLETE URINE ANALYSIS				
Physical Examination				
Colour	Pale Yellow		Pale yellow/Yellow	Visual
Appearance	Clear		Clear	Visual Examination
Specific Gravity	1.020		1.005-1.030	pKa Method
pH	6.0		5.0 - 8.0	Double Indicator
Chemical Examination				
Protein	Negative		Negative	Reflectance Photometry / Sulpho Salicylic Acid Method
Glucose	Negative		Negative	Oxidase Peroxidase / Benedict's Test
Ketones	Negative		Negative	Nitroprusside Reaction / Rothera's Method
Bile Salt	Negative		Negative	Hay's Test
Bile Pigment	Negative		Negative	Fouchets method


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Urobilinogen	0.2	mg/dL	0.2 - 1.0 mg/dL	Strip Reflectance / Modified Ehrlich Reaction
Nitrite	Negative		Negative	Strip Reflectance / Diazonium Compound
Leucocyte Esterase	Negative		Negative	Granulocyte Esterases

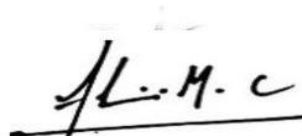
Microscopic Examination (/hpf)

Pus Cell	2-4	cells / HPF	Upto 5 cells / HPF	
Epithelial Cells	2-4	cells / HPF	Upto 5 cells / HPF	Microscopy
Red Blood Cells	Absent	cells / HPF	Absent cells / HPF	Microscopy
Casts	Nil	/ HPF	Nil/ HPF	Microscopy
Crystals	Nil	/ HPF	Nil/ HPF	Microscopy
Amorphous Deposit	Nil	/ HPF	Nil/ HPF	Microscopy
Yeast Cells	Nil	/ HPF	Nil/ HPF	Microscopy
Bacteria	Nil	/ HPF	Nil/ HPF	Microscopy
Other findings	Not Seen		Not seen / HPF	

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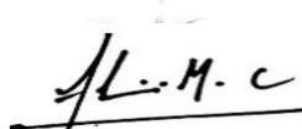


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Sample Receiving Time : 14 Mar, 2026, 07:13 a.m.
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BIOCHEMISTRY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
<u>LIVER FUNCTION TEST SERUM (WITH GGT)</u>				
Bilirubin Total - serum	0.5	mg/dL	0 - 1.2	Diazonium salt (Colorimetric)/Jendrassik
Bilirubin Direct - serum	0.1	mg/dL	0 - 0.20	Diazonium salt(Colorimetric)/Jendrassik
Bilirubin Indirect - serum	0.40	mg/dL	0.3 - 1.0	Calculated
SGOT (AST) - serum	27.4	U/L	10 - 35	IFCC, Enzymatic with PYP
SGPT (ALT) - serum	12.4	U/L	10 - 35	IFCC, Enzymatic with PYP
SGOT/SGPT Ratio- serum	2.21		<2.0	Calculated
Alkaline Phosphatase - serum	98	U/L	35 - 104	Enzymatic IFCC
Protein Total - serum	7.2	g/dL	6.6 - 8.7	Colorimetric-biuret
Albumin - serum	4.3	g/dL	New Born 0-4 days (2.8 - 4.4) 4days -14years (3.8 - 5.4) 14-18years (3.2 - 4.5) Adults (3.5 - 5.2)	Colorimetric-bromocresol green (BCG)
Globulin	2.90	g/dL	2.0 - 4.1	Calculated
A/G Ratio - serum	1.48		1.0 - 2.1	Calculated
GGTP - Gamma GT - serum	16	U/L	5 - 36	Enzymatic-Szasz
Comments :	Outside samples to be correlated with other clinical findings			


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Note : -

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IMMUNOLOGY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
THYROID PROFILE : T3,T4,TSH				
TOTAL TRIIODOTHYRONINE (T3) serum	0.84	ng/mL	0.64-1.52	C M I A
TOTAL THYROXINE (T4) serum	7.67	µg/dL	4.87 - 11.72	C M I A
THYROID STIMULATING HORMONE (TSH) serum	3.47	µIU/mL	0.35 - 4.94	C M I A

Comments : IF NOT ON DRUGS SUGGESTED FT3 & FT4 ESTIMATION

Outside samples to be correlated with other clinical findings

Note : Serum Tri-iodo-thyronine (T3), Thyroxine (T4) and Thyroid stimulating hormone (TSH) form the three components of thyroid screening panel, useful in diagnosing various disorders of the thyroid gland. Primary Hypothyroidism is accompanied by depressed serum T3 and T4 values and elevated serum TSH levels. Although elevated TSH levels are nearly always indicative of Primary Hypothyroidism, rarely they can from TSH secreting pituitary tumors (Secondary hyperthyroidism)To confirm diagnosis - evaluate FT3 and FT4.

Note :

Ashwini M. C.
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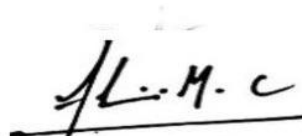


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BIOCHEMISTRY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
LIPID PROFILE				
Sample Type : SERUM	-			
Total Cholesterol - serum	151	mg/dl	Desirable : < 200 Borderline high : 200 - 239 High : > 240	Enzymatic, colorimetric
Triglycerides - serum	45	mg/dl	Normal : <150 Borderline High :150-199 High : 200-499 Very High : ≥500	GPO, Enzymatic colorimetric
HDL Cholesterol -serum	54.3	mg/dl	No risk : >65 Moderate risk : 45 - 65 High risk <45	Direct homogeneous enzymatic colorimetric
Non HDL Cholesterol serum	96.70	mg/dl	Desirable : < 130 Borderline high : 130 - 159 High : ≥ 160	Calculated
LDL Cholesterol -serum	87.70	mg/dL	Desirable : <130 Borderline High : 130-159 ■High : ≥160	Calculated
VLDL Cholesterol -serum	9	mg/dl	Below 30	Calculated
CHOL/HDL Ratio-serum	2.78		Desirable/Low Risk : 3.3 - 4.4 Borderline/Middle Risk : 4.5 - 7.1 Elevated/High Risk : 7.2 - 11.0	Calculated


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Cholesterol LDL/HDL Ratio 1.62
Serum

Desirable/Low Risk : 0.5 - 3.0
Borderline/Middle Risk : 3.1 - 6.0
Elevated/High Risk : >6.1

HDL / LDL Cholesterol Ratio 0.62
-serum

Ratio

>0.40

Calculated

Comments : Outside samples to be correlated with other clinical findings

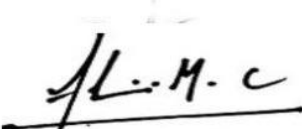
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BIOCHEMISTRY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
<u>KIDNEY BASIC SCREEN - KFT</u>				
Serum Urea	22.0	mg/dl	16.6 - 48.5	Urease/Glutamate dehydrogenase (GLDH)
BUN	10.28	mg/dl	6 - 20	Calculated
Serum Creatinine	0.6	mg/dl	0.50 - 0.90	Jaffé, kinetic colorimetric
Serum Uric acid	3.1	mg/dl	2.4 - 5.7	Enzymatic colorimetric
BUN / Creatinine Ratio	17.13			Calculated
Urea / Creatinine Ratio	36.67			
eGFR	144.78	mL/min	70 - 200	

Comments :

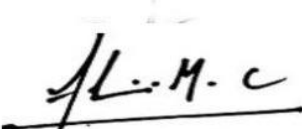
Outside samples to be correlated with other clinical findings

Note : This is an amended report which supersedes the previous report of

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Reported On : 14 Mar, 2026, 12:47 p.m.

Date of Birth : 07-09-1998

IMMUNOLOGY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
<u>VITAMIN - D</u>				
25-OH Vitamin D	11.8	ng/mL	Deficiency <10 ng/mL; Insufficiency 10-29 ng/mL; Sufficiency 30-100 ng/mL; Toxicity >100 ng/mL	CMIA

****END OF REPORT****

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Shruthi Sreenath
Consultant Biochemist

Dr. Ashwini M.C
Consultant Pathologist



These tests were performed at Predlabs Private Limited, Bengaluru

#360, Sri Sai Padma Arcade, 2nd Floor,
Ramagondanahalli, Varthur Main Road,
Whitefield, Bengaluru 560066





Patient Name : **SMITAPRAGYAN RAIGURU**

Age / Sex : 27 years / Female

Ref. Doctor : SELF

Client Name : Medibuddy

Sample ID : 8674902

Patient ID / Billing ID : 806295 / 878879

Specimen Collected by : Medibuddy

Sample Receiving Time : 14 Mar, 2026, 07:13 a.m.

Registration On : 14 Mar, 2026, 07:13 a.m.

Reported On : 14 Mar, 2026, 12:42 p.m.

Date of Birth : 07-09-1998

BIOCHEMISTRY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
<u>SERUM TRANSFERRIN</u>				
Serum Transferrin	317.8	mg/dl	206 - 381	Immunoturbidimetry

****END OF REPORT****

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Date of Birth : 07-09-1998

IMMUNOLOGY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
<u>VITAMIN - B12</u>				
VITAMIN B-12	325	pg/mL	Normal :187-883pg/mL	CMIA

Clinical significance :

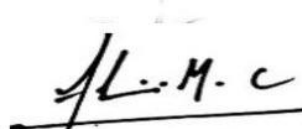
- Vitamin B 12 deficiency frequently causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination and affective behavioral changes.
- Many patients have the neurologic defects without macrocytic anemia. Serum methylmalonic acid (MMA) and homocysteine levels are also elevated in Vit B 12 deficiency states.
- It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias.
- For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Please correlate with clinical conditions.

Limitations:

- The evaluation of macrocytic anemia requires measurement of both vitamin B12 and Folate levels: ideally they should be measured simultaneously.
- Specimen collection soon after blood transfusion can falsely increase Vit B12 levels.
- Patient taking Vit B12 supplementation may have misleading results.
- If Clinical symptoms suggest deficiency, measurement of MMA and Homocysteine should be considered, even if serum B12 concentrations are normal.


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BIOCHEMISTRY

TEST DONE	OBSERVED VALUE	UNIT	BIO. REF. INTERVAL	METHOD
IRON PROFILE				
Iron	76	ug/dL	33 - 193	Colorimetric - FerroZine
UIBC	375	ug/dl	135 - 392	Colorimetric - FerroZine
Total Iron Binding Capacity	451	ug/dL	250- 450	Serum, Ferene
Transferrin Saturation	16.85	%	13-45	Calculated

Comments

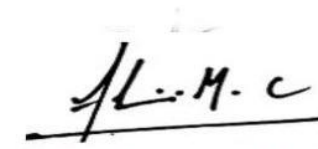
Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron, leads to microcytic hypochromic anemia. The toxic effects of iron are deposition of iron in various organs of the body and hemochromatosis.

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. In iron deficiency anemia, serum iron is reduced and TIBC increases.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

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